BRD Review – Preliminary Comments

Preliminary comments from an accounting perspective, for discussion.

General (both BRDs)

1. **Document structure:** Consider adding standard BRD sections: version control, document owner and approvers, scope and out-of-scope items, definitions/glossary, assumptions, numbered requirements and acceptance criteria. Numbered requirements will help trace each requirement through design and testing.
2. **Approvals & segregation of duties:** Neither document states who may void, refund, approve or pay, or whether amount limits apply. We suggest adding an approval matrix by role and amount. It should also confirm that the person who creates a transaction is not the same person who approves it.
3. **Document status:** Statuses are mentioned in several places (e.g. ORDERED, DRAFT, CONFIRMED, ON HOLD, PARTIALLY_PAID, POSTED, CANCELLED), but no full list is given. Suggest defining the complete set of statuses for each document and the allowed movements between them.
4. **Accounting periods:** Please clarify the posting date used for voids, reversals and refunds when the original transaction falls in an accounting period that is already closed.
5. **Consistency:** The point at which a document can no longer be changed differs: "once paid" (OPD) versus "once approved/posted" (Supplier). The Supplier BRD also says "must not be *silently* modified". Suggest one consistent rule across both documents, and removing the word "silently".
6. **Presentation:** The text-based flow diagrams (arrows) may not display correctly once printed. Consider replacing them with simple tables or proper diagrams.

BRD – OPD Visit

1. **Section 1.1–1.7:** Please define how and when charges collected during the visit are brought together into one invoice. This includes who closes the visit, and whether new charges can be added after the invoice is issued.
2. **General:** The BRD does not cover a patient who leaves without paying, partial payment, deposits, or how an outstanding balance is followed up. Suggest including these, or listing them as out of scope.
3. **General:** Payer types and price adjustments are not covered: insurance, corporate accounts, fee waivers or exemptions, discounts and packages. Please confirm whether these are in scope, as they affect how charges are billed.
4. **Section 2.1 and summary table:** The summary table shows "Void service fee" for nursing and doctor fees, but Section 2.1 only describes voiding registration fees. There is also no void process for procedure or instrument fees once charged but not yet paid. Suggest one void process for any charge raised in error (e.g. wrong patient, wrong visit, duplicate).
5. **Section 3.1 / 3.2:** Refunds are paid "immediately" with no approval step. Suggest adding refund approval and amount limits, and stating the refund method (cash or the original payment method).
6. **Section 1.5:** The process lists dispensing before invoicing and payment, so the patient may receive medicine before paying. Please confirm this is intended, and state what happens if the patient does not pay. Please also note that this creates a separate pharmacy invoice in addition to the end-of-visit invoice.
7. **Section 1.5 / 2.2:** Partial dispensing is not covered. Suggest stating how the undispensed balance of an order is handled (kept open, or cancelled).

8. **Section 1.3 / 1.4 / 5.2:** The billing trigger is described differently in different sections. For example, Section 1.3 says "first vital-sign record", while Section 5.2 says "nursing encounter started". Please align the wording. Please also clarify what happens to the fee if that first record is deleted, and whether a second doctor seen in the same visit is charged.
9. **Section 1.6:** Consumables and implants used in procedures (e.g. sutures, dressings, the implant itself) do not appear to be billed or taken out of stock anywhere. "Billable milestone" in the summary table should also be defined.
10. **Section 2.1:** Please clarify what happens to charges already raised when a visit or patient registration is cancelled.
11. **Section 3.1:** Suggest adding medicine return criteria (e.g. opened packs, cold-chain and controlled drugs), how the refund amount is calculated (original price, net of any discount), and whether returned stock goes back into inventory or is written off.
12. **Section 1.1:** Please confirm whether the patient registration fee is charged once only, or at each registration. Please also state where prices come from (price list, effective dates, tax treatment).
13. **Section 5.1 / 5.5:** "Device ordered" (5.1) does not match Section 1.7, where only the medication is ordered. The audit trail (5.5) should also record when each charge is created, not only adjustments.

BRD – Supplier Delivery, Invoice and Payment

1. **Section 1.1 / 5.2:** The BRD states that a confirmed delivery has no financial effect. Under accrual accounting with perpetual inventory, receiving goods normally increases inventory and records a liability for goods received but not yet invoiced (GRNI). Without this, inventory and liabilities may be understated at month-end when goods arrive before the invoice. Please confirm this accounting policy with Finance. If the current approach is kept, please describe how month-end accruals will be handled.
2. **Section 1.2:** Invoices are matched to the delivery only. Suggest a three-way match (Purchase Order, delivery and invoice), with prices checked against the PO rather than prices entered by receiving staff. Please also define acceptable tolerances, and who may approve an invoice that does not fully match.
3. **Section 1.3:** There is no payment approval step before payment is made. Suggest adding: payment approval and limits; a check that payment does not exceed the amount still owed; payment statuses (draft, approved, posted); and controls over changes to supplier bank details.
4. **Section 1.2:** Suggest an explicit check to prevent the same supplier invoice being recorded twice, for example by checking supplier, invoice number and amount.
5. **Section 2.2 / 3.1:** A "payment reversal" corrects cash/bank only if the payment did not actually go out (e.g. a cancelled cheque or rejected transfer). If the supplier has already received the money, recovering it is a refund received from the supplier. Suggest describing these as two separate processes.
6. **Section 1.2:** Please clarify how invoices relate to deliveries: one invoice covering several deliveries, and one delivery billed across several invoices.
7. **Section 2.1:** Cancelling a confirmed delivery does not address stock already received or used. It also does not cover a delivery already matched to an approved or paid invoice. Suggest blocking cancellation in those cases. Suggest also adding a goods-return-to-supplier / debit note process.
8. **Section 3.3:** Section 3.3 allows the delivery or invoice to be "corrected" when releasing a hold. A supplier invoice should not be edited; the supplier should issue a corrected invoice or a credit note instead. Changes to a confirmed delivery should be controlled and recorded.

9. **Section 3.2:** Please describe how amounts due back from suppliers (a debit balance on the supplier account) are tracked, offset against later invoices, or collected.
10. **General:** Please confirm whether the following are in scope: VAT / withholding tax, foreign currency, payment terms and due dates, early-payment discounts, advance payments to suppliers, and invoices for services that have no delivery.
11. **Sections 4 and 5.6:** In the summary table, the actor "Receiving / Supplier" should be internal staff only. "Generated at: Delivery confirmed" leaves out the draft stage. Suggest adding the bank/payment reference and the PO to the audit trail.